

Dental implants – consultation and treatment

Understanding implants

An implant replaces a missing tooth root. This combined guide takes you from your first questions through consultation and planning to treatment and long-term care. It supplements your personal consultation; examination and shared decision-making determine which steps are appropriate for you.

Implant, abutment and restoration

The implant body is anchored in the jawbone. A connecting part later supports the visible crown, bridge or denture. An implant alone is therefore not a complete tooth. We provide surgical planning and treatment in coordination with your general dental practice, which plans and provides the definitive prosthetic restoration.

Titanium is our established standard. Ceramic implants may be an alternative in selected cases; their properties, limitations and costs need individual discussion. Implant dimensions and number depend on the bone and planned restoration. Tell the radiology team about your implants before an MRI examination.

What can implants support?

- A single missing tooth: one implant can support a crown, usually without preparing healthy neighbouring teeth.
- A larger gap: several implants can support a bridge; not every missing tooth needs its own implant.
- An edentulous jaw: implants can stabilise removable dentures or support a fixed restoration.
- If your dental situation changes, a suitable existing implant may sometimes be incorporated into a new restoration.

Considering alternatives

A bridge, removable denture or, in selected situations, leaving a gap may be alternatives. A bridge can be particularly appropriate when neighbouring teeth already need crowns. Treatment burden, cleaning, adjacent teeth, cost and your preferences all matter. A tooth worth preserving is not removed simply because an implant is available.

Your first consultation

Your first appointment is for examination, discussion and planning, not implant surgery. You do not need to know the specialist techniques beforehand. The appointment does not commit you to treatment. We assess your wishes, suitability, alternatives and likely timescale and costs.

Please bring

- A current medication list, including non-prescription medicines and supplements.
- An allergy record and any existing X-rays, panoramic radiographs or CBCT scans. Do not arrange new scans just in anticipation of the appointment.
- Your referral or your general dentist's questions, and information about the planned restoration.
- Existing dentures or temporary teeth and details of your insurance.

Please tell us beforehand

- Smoking, diabetes (HbA1c if known), periodontitis, previous implant loss or healing problems.
- Anticoagulants or antiplatelet medicines such as aspirin, warfarin/phenprocoumon or direct oral anticoagulants.
- Current or previous bisphosphonate or denosumab treatment: medication, duration and last dose if known.
- Immunosuppression, steroids, chemotherapy or radiotherapy, especially to the head and neck; significant cardiovascular conditions and recent major procedures.
- Allergies, possible pregnancy, and significant tooth grinding or clenching.

Do not stop or change regular medication yourself. Any adjustment depends on the drug, procedure and underlying condition and is coordinated with you and, where needed, your treating doctor. INR testing is not appropriate for every anticoagulant.

Your questions shape the plan

What bothers you? Would you prefer fixed or removable teeth? How important are treatment time, anxiety, temporary teeth and cost? Bring your questions. Age alone does not determine suitability: health, completed jaw growth, ability to maintain hygiene and treatment goals are important.

Digital implant planning

We plan backwards from the desired restoration. First, the future tooth position is digitally simulated. This determines a suitable implant position and whether additional bone is needed. This approach is called backward planning.

From the prosthetic setup to surgery

- Examination and scans or models record teeth, soft tissues, bite and available space.
- The crown, bridge or denture is planned with your general dental practice, for example using the opposite side or a digital tooth library.
- Appropriate radiographs help assess bone, neighbouring teeth, the lower jaw nerve and the maxillary sinus where relevant.
- When needed, individually manufactured guides help transfer the plan to surgery. A guide does not replace surgical judgement or eliminate all risks.

Is CBCT always needed?

No. Existing or conventional radiographs may be sufficient in straightforward situations. We use CBCT when the additional three-dimensional information materially supports diagnosis or safe planning. Benefits, radiation exposure and additional costs are explained before the scan.

If bone is insufficient

Small defects can sometimes be augmented at implant placement. A larger height or width deficiency often requires a separate bone augmentation procedure and healing period first. A sinus lift may be appropriate in the back of the upper jaw. A bridge, denture or a different implant plan may sometimes be an alternative to extensive grafting.

The technique is chosen after assessment. You then receive the information relevant to your procedure; you do not need to read about every grafting technique in advance.

Your treatment journey

1 · Preparation and timing

Before surgery, inflammation and periodontitis are treated where needed, hygiene is improved and temporary teeth are planned. Following extraction, we often arrange the initial implant assessment after about eight weeks of healing. This is a guide: actual implant timing depends on soft tissues, bone, infection and the restoration plan. Immediate placement is a separate pathway for suitable cases.

2 · Implant placement

Under effective anaesthesia, the implant site is prepared, the implant is inserted and the wound is closed as appropriate. A planned small graft may be performed at the same time. Straightforward placement may take around 20–30 minutes; multiple implants and additional procedures take longer. Unexpected findings may require a change of plan or staged treatment.

3 · Protected healing

In our usual approach the implant initially heals beneath the gum. Bone healing often takes about three to four months, and longer after grafting or with less favourable conditions. Avoid biting onto the surgical site or pressure from temporary teeth. Immediate loading is not a standard promise.

4 · Uncovering and restoration

Once healing is confirmed, a submerged implant is exposed in a small second procedure and a healing abutment is fitted. After soft tissue healing, often about four weeks, your general dental practice takes a scan or impression and provides the restoration. Sutures are usually removed after one to two weeks, depending on the procedure and wound; your appointment is arranged individually.

A straightforward restoration often takes about five to six months from implant placement to the final crown. A separate major graft may add several months. These are estimates, not guaranteed dates.

Temporary teeth, work and travel

Temporary teeth are planned with your general dentist and must keep pressure off the surgical site. Additional temporary anchorage may be considered in special cases. Work, exercise and travel depend on the procedure and healing. Allow time for reviews and discuss travel with limited access to medical care beforehand.

Anaesthesia, risks and costs

Your treatment, personal risks, alternatives and costs are discussed before you decide. Successful healing or lifelong function cannot be guaranteed.

Anaesthesia and comfort options

Local anaesthesia is standard. You remain awake and may feel pressure or movement. If you feel pain, we stop and give additional anaesthetic. Sedation and general anaesthesia require a separate assessment and consent discussion.

Method	Procedure and important rules	Costs
Local anaesthesia Standard	The area is numbed; you remain awake. Public transport home is possible; individual fitness to travel is assessed.	Included in the surgical treatment plan.
Oral sedation Lorazepam (Tavor)	A tablet usually about 60 minutes beforehand reduces anxiety without guaranteeing sleep. Vital signs are monitored. An escort must collect you from the practice. No driving for 24 hours.	About €80 privately paid.
Intravenous sedation	Not routinely offered for implant placement in our practice: small components and coughing or gagging may create an aspiration risk. Only after individual assessment with the anaesthesia team. If agreed: Tuesdays, monitored recovery, collection from the practice by an escort mandatory; no driving for 24 hours.	Exceptional cases only; agree costs beforehand.
General anaesthesia	Deep sleep with a secured airway, usually a breathing tube passed through the nose. Anaesthesia team available on Tuesdays, with monitored recovery. An escort must collect you from the practice; no driving for 24 hours.	Time-dependent; usually about €500–800 additional anaesthesia costs.

Local anaesthesia is sufficient for most straightforward implant procedures. Follow the anaesthesia team's specific fasting instructions for IV sedation or general anaesthesia. Our usual schedule is no food after 10 pm and no drinks after 6 am; different individual instructions take priority. Take medication only as agreed.

Anaesthetic risks

Local anaesthesia may cause circulatory reactions, bruising, allergic reactions and rarely nerve irritation. Sedation and general anaesthesia can affect breathing and circulation and cause nausea, vomiting or aspiration; serious complications are rare but possible. Monitoring, appropriate selection, fasting and escort arrangements are therefore important.

Possible risks of implant treatment

- Pain, swelling, bruising and restricted mouth opening; bleeding, infection, wound opening and delayed healing.
- Failure to integrate or later implant loss. Another restoration may be possible after healing; additional treatment or a different solution may be needed.
- Damage to neighbouring teeth or nerves: numbness, tingling or altered sensation in the lip, chin or tongue may be temporary and rarely permanent. Other sensory structures may be affected depending on the approach.
- In the upper jaw: sinus opening or inflammation, including a mouth–sinus communication; very rarely an implant may be displaced into the sinus.
- Later problems include gum recession, visible implant surfaces, inflammation, loose screws or damage to the implant or restoration. Implant or jaw fractures and swallowing or inhaling small components are very rare possibilities.
- Additional grafting has procedure-specific risks, such as graft loss or problems at a bone donor site.

Smoking, inadequate hygiene, untreated periodontitis, poorly controlled diabetes and certain medicines increase risks. With bisphosphonates/denosumab or previous radiotherapy, jaw osteonecrosis is a particular concern. We discuss the personal significance of these factors before treatment.

Planning costs transparently

Initial consultation: up to €150 depending on complexity and investigations; our implant consultation is offered as a privately paid service. **CBCT if needed:** €180–280 depending on the diagnostic question. Charges are explained beforehand.

An individual cost plan covers implant placement, any grafting, uncovering and optional anaesthesia. Your general dentist charges separately for abutments, crowns, bridges or dentures. The definitive plan follows adequate diagnostic assessment; CBCT is not automatically a prerequisite for every patient.

German statutory insurance generally does not cover implant surgery; rare legally defined exceptions require separate assessment. A diagnosis-based subsidy may be available for the restoration. Private insurance, supplementary insurance and civil-service reimbursement depend on individual cover and applicable rules. Submit plans for assessment before treatment.

Implant loss can occur despite appropriate treatment. Its cause, further treatment and possible additional costs are assessed individually; neither free replacement nor a finding of treatment error follows automatically.

Long-term care and maintenance

Protect healing

Do not bite onto the freshly operated ridge. Eat soft food and avoid pressure from hard food or temporary teeth. Clean the other teeth carefully; follow our instructions for the wound and any prescribed antiseptic rinse. Avoid smoking, attend reviews and contact us early if problems arise. Detailed postoperative instructions are provided for recovery.

Home care and professional reviews work together

Implants can function for many years. Longevity depends on the starting situation, treatment, restoration, hygiene and maintenance. Clean thoroughly every day with a toothbrush and suitable interdental aids. Brushes or special floss are selected for your restoration. Professional cleaning and dental reviews are tailored to risk; a water flosser does not replace mechanical cleaning.

After our surgical implant treatment, we review bone, soft tissues, hygiene, cleanability and the stability of the restoration annually for the first two years. If conditions remain stable, surgical review intervals may then be extended. This does not replace ongoing care by your general dentist. Inflammation, increased risk or previous peri-implantitis treatment require more frequent review. Radiographs are taken when indicated.

Our surgical implant review costs up to €110 depending on the work and examination required. This is not a price for any peri-implantitis treatment.

When to contact us

Increasing pain or swelling, persistent bleeding, pus, fever, new or persistent altered sensation and a loose implant or restoration need assessment. A loose crown may also reflect a loose screw: stop loading it and arrange prompt review.

Practice: 06151 786540. After surgery outside consultation hours: +49 151 10437450. If unavailable: dental emergency service 01805 60 70 11. For a life-threatening emergency, shortness of breath, severe difficulty swallowing or rapidly increasing swelling of the floor of the mouth, neck or tongue: 112.